

Medical Prescription & Care Plan

Date: 17 Sep 2025

Patient Information

Name: Dr. Aruna Kulatunga Age: 63 Sex: Male

Conditions: Type 2 Diabetes Mellitus, Hypertension, Chronic Kidney Disease Stage 3b, Severe Dyslipidemia, Hyperuricemia

Clinical Observations (most recent key labs)

- eGFR ~31 mL/min/1.73m² (CKD 3b); Creatinine ~2.3 mg/dL
- HbA1c ~8.4%; Fasting glucose ~123 mg/dL; recurrent post-meal spikes
- LDL-C 256 mg/dL (very high); Total Cholesterol 355 mg/dL; TG 191 mg/dL; HDL 61 mg/dL
- Marked albuminuria (A3 level on prior UACR)
- Uric Acid 8.5 mg/dL

Note: This document summarizes a proposed prescription and monitoring plan; final prescribing should be confirmed by the treating physician after clinical examination.

Medication Plan (generic + Sri Lanka brand examples)

Telmisartan (ARB)

- Dose: 80 mg | Route/Frequency: PO once daily | Timing: Morning
Brand (SL): Telday 80 (Cipla SL)

- Dose: 10 mg | Route/Frequency: PO once daily | Timing: Morning
Brand (SL): Jardiance 10 mg
Notes: Hold during dehydration/acute illness; monitor renal function.

- Dose: 1.5 mg | Route/Frequency: PO once daily | Timing: Morning
Brand (SL): Natrilix SR
Notes: Replace Hydrochlorothiazide 50 mg to reduce urate impact; check Na⁺/K⁺ 1-2 weeks after change.

- Dose: 10 mg | Route/Frequency: PO once daily | Timing: Night
Brand (SL): Cilacar 10
Notes: May cause ankle edema; monitor.

- Dose: 75 mg | Route/Frequency: PO at bedtime | Timing: Night
Brand (SL): Lyrica / generic pregabalin
Notes: Avoid dose escalation at eGFR ~31; caution drowsiness/imbalance.

- Dose: 75 mg | Route/Frequency: PO once daily | Timing: Night (if continued)
Brand (SL): Ecosprin 75
Notes: For primary prevention in age ≥60, continuation should be individualized; discuss with clinician.

- Dose: 10 mg | Route/Frequency: PO once daily | Timing: Any time (same time daily)
Brand (SL): Kerendia

Notes: Start if K+ ≤ 4.8 and eGFR ≥ 25 ; recheck K+/creatinine at 4 weeks; titrate to 20 mg if tolerated.

Insul

- Dose: Per clinician (e.g., ~25 units pre-breakfast +/- pre-dinner) | Route/Frequency: SC | Timing: 20-30 min before meals

Brand (SL): Mixtard 30/70 (Novo Nordisk)

Notes: Consider adding small pre-meal rapid insulin or switching to basal-bolus if post-meal spikes persist.

Lipid

- Dose: Atorvastatin 20 mg + Ezetimibe 10 mg | Route/Frequency: PO nightly | Timing: Night

Brand (SL): Atorva (Astron) + generic ezetimibe

Notes: If LDL ≥ 100 at 4-8 weeks, increase atorvastatin to 40 mg (keep ezetimibe). Target LDL < 70 mg/dL.

Lipid

- Dose: Ezetimibe 10 mg | Route/Frequency: PO once daily | Timing: Night

Brand (SL): Generic ezetimibe

Notes: If LDL remains above goal, consider PCSK9 inhibitor via specialist (availability/coverage dependent).

As-n

- Aciclovir 800 mg (e.g., Herperax 800) as per clinician for HSV flares.

- OTC supplements optional: multivitamin (e.g., Swisse Men 55+), glucosamine HCl/krill oil; avoid high-sodium forms.

Testing & Monitoring Plan

- Baseline now (17 Sep 2025): Serum Potassium, Creatinine/eGFR, Sodium; Lipid profile; HbA1c; Urine ACR; LFTs.

- After finerenone start: Recheck K+ & Creatinine/eGFR at 15 Oct 2025, 12 Nov 2025, 10 Dec 2025; then every 3-4 months or after dose changes.

- Lipids: 4-8 weeks after starting/up-titrating statin/ezetimibe, then every 3-12 months.

- Diabetes: HbA1c every 3 months; SMBG focusing on fasting and 2-hr post-meal; consider CGM if available.

- Kidney: UACR every 3 months; avoid NSAIDs; review nephrology.

- Eyes & feet: Annual retinal exam; foot exam every visit; daily self-inspection.

- BP: Home log with standardized technique; aim SBP ~110-130 without DBP < 60 .

Key Warnings / Interactions

- Finerenone: risk of hyperkalemia; do not combine with ACEi+ARB dual blockade; review CYP3A4 inhibitors; limit grapefruit.

- Atorvastatin: avoid grapefruit juice; watch for myalgias; check LFTs if symptomatic.

- SGLT2i (empagliflozin): hold during acute illness/dehydration; maintain hydration.

- Insulin: risk of hypoglycemia; ensure meal timing and dose review with clinician.

Prescriber to review and countersign:

Physician Name/Stamp/Signature: _____ SLMC #: _____ Date: _____